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REPORT

PT Name	: **MRS. LEENA GOHIR**	**Age :** 40 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 26-08-2026 04:47 PM	
Reg No	: **HL3301827530 / HYMH/MH410**	**Received on** : 26-08-2026 05:42 PM	
Barcode	: **HEM070864**	**Reported on** : 26-08-2026 09:13 PM	**SAMPLE :** Serum
INV	: Liver Function Test - Aarogya365-2		

ALBUMIN/GLOBULIN RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ALBUMIN - SERUM Method: PHOTOMETRY	4.67	gm/dL	3.2 - 5.2
GLOBULIN Method: PHOTOMETRY	3.22	gm/dL	2.5 - 3.4
Albumin/Globulin Ratio Method: Calculated	1.45	Ratio	0.9 - 2.0

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MRS SHIVANI PARDESHI**	**Age :** 29 Y | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. J.J.JAIN**	**Registered on :** 31-08-2026 04:02 PM	
Reg No	: **HL2401832342 / MH090**	**Received on** : 31-08-2026 06:33 PM	
Barcode	: **HEN178812**	**Reported on** : 31-08-2026 08:10 PM	
INV	: DENGUE IgG,IgM-Rapid		**SAMPLE :** Serum

DENGUE IGG,IGM-RAPID

TEST DESCRIPTION	RESULT	UNITS	RANGES
Dengue Antibody, IgG	NEGATIVE	.	.-NEGATIVE
Dengue Antibody, IgM	NEGATIVE	.	.-NEGATIVE

Comments:

Dengue viruses belong to the family Flaviviridae and have 4 subtypes (1-4). Dengue virus is transmitted by the mosquito Aedes aegypti and Aedes albopictus, widely distributed in Tropical and Subtropical areas of the world. Dengue is considered to be the most important arthropod borne viral disease due to the human morbidity and mortality it causes. The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.

INTERPRETATION:

ANTIBODY	RESULT (INDEX)	REMARKS
Dengue IgM	Negative	No detectable IgM antibody. Result does not rule out Dengue infection. Additional sample to be tested after 7-14 days if infection is suspected.
Dengue IgG	Negative	No detectable IgG antibody indicating a presumptive evidence that the patient does not have secondary Dengue infection.

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REPORT

PT Name : **MR. NARAYAN DHANEGAV**
Ref By : **DR. LIFE CARE PATHOLOGY**
Reg No : **HL3201828753 / MH713**
Barcode : **HEN143904**
INV : Insulin Antibodies, Serum

Age : 80 Year | **Sex :** Male
Registered on : 27-08-2026 10:05 PM
Received on : 28-08-2026 02:55 AM
Reported on : 28-08-2026 04:59 AM

SAMPLE COLLECTED AT :
LIFE CARE PATHOLOGY

SAMPLE : Serum

INSULIN ANTIBODIES, SERUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Insulin antibodies, serum Method: Enzymatic immunoassay(EIA)	5.89	U/mL	. - . <12: NORMAL 12-18: EQUIVOCAL >18: POSITIVE

~~End of report~~

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REPORT

PT Name	: **MR. SACHIN KORDE**	**Age :** 26 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** RAMKRUSHNA PATH LAB RISOD
Ref By	: **DR. PRASHANT ATOLE SIR MBBS.,FCPS.**	**Registered on :** 29-08-2026 09:18 PM	
Reg No	: **HL9701830650 / MH141/MH343**	**Received on** : 01-09-2026 06:11 PM	
Barcode	: **HEN160702**	**Reported on** : 04-09-2026 01:06 PM	
INV	: JAK2 Mutation Test		**SAMPLE :** EDTA Blood

JAK2 MUTATION TEST

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
JAK2 V617F Qualitative PCR*	**Wild**		

Method : Real Time Polymerase chain Reaction (PCR)

The JAK2 genetic mutation is observed at high frequency in Myeloproliferative disorders(MPD) patients. After recent studies have revealed the relationship between MPD and the JAK2 point mutation, many assays have been developed to screen for the JAK2 mutation (JAK2 V617F) for diagnosis of MPD. The JAK2 V617F mutation can be found in 65~97% of polycythemia vera cases, 23~57% in thrombocythemia cases, and 43~50% in idiopathic myelofibrosis cases

Interpretation :

Limit of detection: 10ng/ul

"Wild" : No mutation in JAK2 gene
"Heterzygous mutant" : partial mutancy in JAK2 gene
"Homozygous mutant": 100% mutancy in JAK2 gene

Methodology Details :
* DNA is extracted from samples by US FDA approved Automatic Extraction machine based on magnetic bead technology.
* Purified DNA is then Amplified and quantified using CE-IVD approved Real time PCR.
* Extraction and Amplification controls (IC) are incorporated in each run to ensure more accurate and precise detection of mutation.

~~End of report~~

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REPORT

PT Name : **ARCHANA ROHIRA**
Ref By : **DR. SELF**
Reg No : **HL3001827421 / RAJ04**
Barcode : **HEN143633**
INV : KAPPA / LAMBDA LIGHT CHAINS, FREE, SERUM

Age : 53 Year | **Sex :** Female
Registered on : 26-08-2026 03:48 PM
Received on : 26-08-2026 04:24 PM
Reported on : 27-08-2026 04:19 PM

SAMPLE COLLECTED AT :
BLUECLIFFE LABORATORY

SAMPLE : Serum

KAPPA / LAMBDA LIGHT CHAINS, FREE, SERUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Serum Free Kappa (Light Chain) Method: CLIA	12.5	mg/L	4.78 - 18.22
Serum Free Lambda (Light Chain) Method: CLIA	**2.14**	mg/L	3.71 - 31.7
Serum Free Kappa/ Lambda Ratio Method: CALCULATED	**5.84**	Ratio	0.241 - 1.173

Interpretation :
There are two types of light chains: lambda and kappa light chains. A free light chains test measures the amount of lambda and kappa free light chains in the blood. If the amount of free light chains is higher or lower than normal, it can mean you have a disorder of the plasma cells.

~~End of report~~

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REPORT

PT Name : **RUKMINI MADANE**
Ref By : **DR. SHREE JAGDAMBA LAB**
Reg No : **HL1901830776 / PUN104**
Barcode : **Hen183604**
INV : SPUTUM FOR GRAM STAIN

Age : 75 Year | **Sex :** Female
Registered on : 30-08-2026 05:12 AM
Received on : 30-08-2026 09:13 AM
Reported on : 30-08-2026 10:09 PM

SAMPLE COLLECTED AT :

SAMPLE : SPUTUM

SPUTUM FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SAMPLE TYPE :	SPUTUM	.	. - .
MICROSCOPY :	NO MICRO-ORGANISM SEEN	.	. - .

~~End of report~~

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REPORT

PT Name	: **CHRISTINA PICARDO**	**Age :** 54 Y | **Sex :** Female	**SAMPLE COLLECTED AT :** RR BLOOD COLLECTION CENTRE
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 12:46 PM	
Reg No	: **HL6901830080 / MH206**	**Received on** : 31-08-2026 10:58 AM	
Barcode	: **HEL037393**	**Reported on** : 04-09-2026 04:17 PM	**SAMPLE :** Serum
INV	: ALLERGY FOOD AND NON VEG		

ALLERGY FOOD AND NON VEG

TEST DESCRIPTION	RESULT	UNITS	RANGES
SERUM IMMUNOGLOBULIN - E (IgE) Method: CLIA	1025.3	IU/mL	0-3 Y : 0 - 46 3-16 Y : 0 - 280 ADULTS-. Y : 0 - 100

ALLERGY SCREENING TEST (BY ELISA)

NORMAL VALUE FOR ALL ALLERGEN IS LESS THEN	**0.35 IU / ml**	**NEGATIVE**
	0.35 -- 1.0 IU / ml	**LOW**
	1.1 -- 5.0 IU / ml	**MODERATE**
	5.1 -- 25.0 IU / ml.	**HIGH**

< 0.35	**No specific antibodies detected.**
0.35 -- 1.0	**Low antibody titer, existing sensitisation, frequently with clinical symptoms in the upper range of class**
1.1 -- 5.0	**Significant antibody titer, clinical symptoms usually present**
5.1 -- 25.0	**High antibody titer, almost always with clinical symptoms**

Allergen Detected

Food Veg	**BANANA,DAL URAD,MSG (AJINOMOTO),MUSHROOM,YEAST**
Food Non Veg	**CHICKEN MEAT**

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REPORT

PT Name : **CHRISTINA PICARDO**
Ref By : **DR. SELF**
Reg No : **HL6901830080 / MH206**
Barcode : **HEL037393**
INV : ALLERGY FOOD AND NON VEG

Age : 54 Y | **Sex :** Female
Registered on : 29-08-2026 12:46 PM
Received on : 31-08-2026 10:58 AM
Reported on : 04-09-2026 04:17 PM

SAMPLE COLLECTED AT :
RR BLOOD COLLECTION CENTRE

SAMPLE : Serum

FOOD VEG

ALMOND			0.15	ANISEED			0.22
APPLE			0.10	ARECANUT			0.18
BAJARI			0.16	**BANANA**		+	0.47
BARLEY			0.14	BLACK PEPPER			0.29
BRINJAL			0.23	CABBAGE			0.14
CAPSICUM			0.27	CARDIMUM			0.27
CARROT			0.22	CASHEWNUT			0.23
CAULIFLOWER			0.20	CHEESE			0.27
CHILLY			0.13	CINNAMON			0.22
CLOVE			0.14	COCOA			0.14
COCONUT			0.19	COFFEE			0.18
CORIANDER			0.19	COTTON SEED			0.21
CUCUMBER			0.27	CUMMIN			0.20
CURD			0.30	DAL ARHAR			0.13
DAL CHANA			0.14	DAL MASOOR			0.26
DAL MOONG			0.12	DAL RAJMA			0.30
DAL URAD		+	0.87	DATES			0.30
EGGS			0.10	FENUGREEK			0.30
GARLIC			0.24	GINGER			0.21
GRAPES			0.21	GROUNDNUT			0.14
GUVAVA			0.10	HONEY			0.21

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REPORT

PT Name : CHRISTINA PICARDO
Ref By : DR. SELF
Reg No : HL6901830080 / MH206
Barcode : HEL037393
INV : ALLERGY FOOD AND NON VEG

Age : 54 Y | **Sex :** Female
Registered on : 29-08-2026 12:46 PM
Received on : 31-08-2026 10:58 AM
Reported on : 04-09-2026 04:17 PM

SAMPLE COLLECTED AT :
RR BLOOD COLLECTION CENTRE

SAMPLE : Serum

JAGGERY			0.19	KATHA			0.27
LADY S FINGER			0.28	LEMON			0.27
MAIZE			0.27	MANGO			0.30
MELON			0.25	MILK			0.23
MSG (AJINOMOTO)		+	1.54	**MUSHROOM**		+	1.65
MUSTARD			0.28	NUTMEG			0.22
OAT			0.22	ONION			0.21
ORANGE			0.20	PAPAYA			0.23
PEAS			0.15	PINEAPPLE			0.28
PISTA			0.25	POTATO			0.20
RADISH			0.10	RICE			0.10
SALT			0.10	SESAME			0.13
SOYABEAN			0.13	SPINACH			0.20
SQUASH			0.28	STRAW BERRY			0.11
SUGAR			0.24	SUNFLOWER			0.18
TAMARIND			0.29	TEA			0.16
TOBACCO			0.21	TOMATOES			0.16
TURMERIC			0.21	VINEGAR			0.20
WALNUT			0.20	WHEAT			0.14
YEAST		+	0.77				

FOOD NON VEG

BEEF			0.10	**CHICKEN MEAT**		+	1.58

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REPORT

PT Name	: **CHRISTINA PICARDO**	**Age :** 54 Y | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 12:46 PM	RR BLOOD COLLECTION CENTRE
Reg No	: **HL6901830080 / MH206**	**Received on** : 31-08-2026 10:58 AM	
Barcode	: **HEL037393**	**Reported on** : 04-09-2026 04:17 PM	
INV	: ALLERGY FOOD AND NON VEG		**SAMPLE :** Serum

CRAB			0.14
PORK			0.18
SHRIMP			0.24

MUTTON			0.10
PRAWN			0.11

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REPORT

PT Name	: **HARISHCHANDRA SAMBARE**	**Age :** 70 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. JIJAU HOSPITAL, RATNAGIRI**	**Registered on :** 27-08-2026 09:24 PM	JIJAU SANSTHA RATNAGIRI
Reg No	: **HL3801828717 / MH004/012/DEV55**	**Received on** : 28-08-2026 08:02 AM	
Barcode	: **HEE176937**	**Reported on** : 07-09-2026 05:06 PM	
INV	: Blood C/s Bactec Aerobic		**SAMPLE :** Heparin

BLOOD C/S BACTEC AEROBIC

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

Preliminary Report - No bacterial growth was observed after 48 hours of aerobic incubation.
Secondary Report - No bacterial growth was observed after 72 hours of aerobic incubation.
Organism Isolated - Nil
Antibiogram - .
Sensitive to - Nil
Intermediate to - Nil
Resistant to - Nil
FINAL REPORT - No Bacterial Growth Was Observed After 1 week of Aerobic Incubation.
NOTE -

~~End of report~~

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REPORT

PT Name	: **MR. ANAND PRAKASH PANDEY**	**Age :** 64 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 01-09-2026 04:01 PM	
Reg No	: **HL9801832602 / HYMH/KDV/H1886**	**Received on** : 01-09-2026 06:03 PM	
Barcode	: **HEN173356**	**Reported on** : 01-09-2026 07:24 PM	**SAMPLE :** Fluoride Plasma-02
INV	: Glucose PP (Blood Sugar Post Prandial)		

GLUCOSE PP (BLOOD SUGAR POST PRANDIAL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Glucose, Post Prandial (PP) Method: GOD-POD	**189.8**	mg/dl	74 - 140

Interpretation :

When we eat food, the pancreas (an organ that sits between the stomach and the spine) goes to work, releasing enzymes that help to break down food and hormones that help the body handle the influx of glucose. One of these hormones is insulin, and it plays a key role in managing glucose levels in the blood.

And here is where things can go wrong. If the pancreas doesn't make enough insulin — or stops making it altogether, in the case of type 1 diabetes — glucose levels in the blood can rise too high. Another scenario is that the pancreas makes enough insulin but the cells have trouble using it properly, causing blood glucose levels to rise. This is called insulin resistance and is the hallmark of type 2 diabetes.

In the short term, high blood glucose levels can make you feel downright bad. Thirst, frequent trips to the bathroom, fatigue, and weight loss are all symptoms of high blood glucose (hyperglycemia). If not treated, more serious issues can occur, such as diabetic ketoacidosis (DKA). Chronic high blood glucose levels can lead to complications such as heart, kidney and eye disease, as well as nerve damage. So, it's all about the blood glucose.

~~End of report~~

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REPORT

PT Name	: **SANA**	**Age :** 30 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 31-08-2026 11:40 PM	
Reg No	: **HL9701832573 / RAJ41**	**Received on** : 01-09-2026 04:10 AM	
Barcode	: **HEL002899**	**Reported on** : 01-09-2026 07:17 AM	**SAMPLE :** Fluoride Plasma-03
INV	: Glucose R (Blood Sugar Random)		

GLUCOSE R (BLOOD SUGAR RANDOM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Blood Glucose , Random Method: GOD-POD	**85.5**	mg/dl	90 - 140

Interpretation :

A random blood sugar test checks your blood glucose at a random time of day. A level of 200 mg/dL or higher is a sign that you have diabetes.

~~End of report~~

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REPORT

PT Name : **RINALI PALAV**
Ref By : **DR. SELF**
Reg No : **HL3901832179 / MH726**
Barcode : **FA066720, FG730791, FG712581**

Age : 31 Year | **Sex :** Female
Registered on : 31-08-2026 02:45 PM
Received on : 31-08-2026 05:05 PM
Reported on : 31-08-2026 07:40 PM

SAMPLE COLLECTED AT :
DAAKSH VICARE+

INV : GLUCOSE TOLERANCE TEST (GTT) - 3 SAMPLES (75 Gm)

SAMPLE : Fluoride Plasma-01, Fluoride Plasma-02, Fluoride Plasma-03

GLUCOSE TOLERANCE TEST (GTT) - 3 SAMPLES (75 GM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GLUCOSE, FASTING (F) Method: GOD-POD	78.41	mg/dl	0 - 95
GLUCOSE (1HRS) Method: GOD-POD	169.18	mg/dl	0 - 180
GLUCOSE (2 HRS) Method: GOD-POD	90.46	mg/dl	0 - 155

Interpretation

You will be required to fast for at least eight hours before you take the test. This means you will need to go without any food or any drink besides water. Since the test is often scheduled for the morning, most of your fasting can happen while you sleep the night before.

You can generally follow a normal diet up until the time you begin fasting. Your doctor may ask you to make sure you consume at least 150 grams of carbohydrates each day in the three days before your glucose tolerance test.

If you take any prescription medications or over-the-counter drugs, be sure to discuss these with your doctor before you take the test. Your doctor can let you know whether any of these might alter the results of the test. They can also advise you whether you will need to stop taking any medications during the time you are fasting.

To ensure the most reliable results, ask your doctor for any specific pretest preparations and carefully follow those instructions on the days leading up to your glucose tolerance test.

~~End of report~~

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REPORT

PT Name	: **SABIYA PARVEEN**	**Age :** 28 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. DR. RAKESH PANDEY**	**Registered on :** 31-08-2026 04:31 PM	6387305523
Reg No	: **HL7901832385 / MH457**	**Received on** : 05-09-2026 01:01 PM	
Barcode	: **HEN122871**	**Reported on** : 10-09-2026 02:57 PM	
INV	: Histopathology (s)		**SAMPLE :** Tissue

HISTOPATHOLOGY (S)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

BIOPSY-14978/26

NOS-ENDOMETRIAL TISSUE FOR HPE.

GROSS- RECEIVED TINY GRAY BROWN TISSUE BIT MEASURING 0.5X0.5 CM, ,ALL EMBEDED IN ONE BLOCK.

MICROSCOPIC PICTURE-SMEARS STUDIED SHOWS CYSTICALLY DILATED GLANDS (> 2X NORMAL SIZE) RANDOMLY INTERSPERSED AMONG PROLIFERATIVE ENDOMETRIAL GLANDS
DILATED GLANDS WITH IRREGULAR SHAPE (BRANCHED, CONVOLUTED, SCALLOPED OUTER CONTOURS)
> 10% OF OVERALL GLANDS
RELATIVELY NORMAL GLAND TO STROMA RATIO (GLANDS OCCUPY < 50% OF THE SURFACE AREA)
METAPLASTIC CHANGES PRESENT, INCLUDING TUBAL METAPLASIA,
STROMAL HEMORRHAGE AND BREAKDOWN PRESENT.
LACK OF CYTOLOGIC ATYPIA.
BACK TO BACK GLANDULAR ARRANGEMENT ,GLAND IN GLAND ARRANGEMENT , WITH INCREASED GLAND TO STROMA RATIO, 2:1. WITH NECROSIS , IN FEW AREAS.

IMPRESSION- SUGGESTIVE OF PROLIFERATIVE PHASE WITH DISORDERED PROLIFERATION .

COMMENT- KINDLY CORRELATE CLINICALLY.

~~End of report~~

Dr. NAMARATA SHARAN (MD PATH)
Consultant Pathologist (Reg. 41002)

HY PATHO LAB
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PROCESSED AT :
HY PATHO LAB RANCHI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **SABIYA PARVEEN**	**Age :** 28 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. DR. RAKESH PANDEY**	**Registered on :** 31-08-2026 04:31 PM	6387305523
Reg No	: **HL7901832385 / MH457**	**Received on** : 05-09-2026 01:01 PM	
Barcode	: **HEN122871**	**Reported on** : 10-09-2026 02:57 PM	
INV	: Histopathology (s)		**SAMPLE :** Tissue

HISTOPATHOLOGY (S)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

BIOPSY-14978/26

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IMPRESSION- SUGGESTIVE OF PROLIFERATIVE PHASE WITH DISORDERED PROLIFERATION .

COMMENT- KINDLY CORRELATE CLINICALLY.

~~End of report~~

Dr. NAMARATA SHARAN (MD PATH)
Consultant Pathologist (Reg. 41002)

PATHO LAB
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HY PATHO LAB MUMBAI

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **VITTHAL KADAM**	**Age :** 75 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. AABU TAYSHETE**	**Registered on :** 29-08-2026 11:29 PM	INDIPATHLAB
Reg No	: **HL1901830694 / MH004/7/MH00K**	**Received on** : 31-08-2026 03:43 PM	
Barcode	: **HED063058**	**Reported on** : 05-09-2026 10:18 AM	
INV	: Pus C/s BACTEC Aerobic		**SAMPLE :** PUS

PUS C/S BACTEC AEROBIC

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

Preliminary Report - No bacterial growth was observed after 48 hours of aerobic incubation.
Secondary Report - No bacterial growth was observed after 72 hours of aerobic incubation.
Organism Isolated - Nil
Antibiogram - .
Sensitive to - Nil
Intermediate to - Nil
Resistant to - Nil
FINAL REPORT - No Bacterial Growth Was Observed After 1 week of Aerobic Incubation.
NOTE -

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)